



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : سماح عبدالرازق محمود صالح

المحافظة : القاهرة الحي / المدينة : المقطم

العنوان : ..

رقم البطاقة : 27709140104681 رقم التليفون : 01279548360

تاريخ تسجيل الإستمارة : 19-09-2026

العمر (تاريخ الميلاد) : 14-09-1977 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج الالم ومضادات القئ لمرضي

الاورام المنتشر [14456]

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... /د. /..... /د. /..... /د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: سماح عبدالرازق محمود صالح National ID: 27709140104681	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حمید بالثدي و تحتاج الي Pin Medication And Anti Emetics
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

٢٠٢٤/٠٤ ٢٧٧٠٩١٤٠١٠٤٦٨١

بدون عمل

انثى

مسترجعة

عبدالرحمن كمال عبدالرحمن محمد ابراهيم

البطاقة سارية حتى ٢٠٣١/٠٤/١٤

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جمهورية دولة فلسطين

مقاطعة جنين - المنطقة

ساح

عبدالرزاق محمود صالح

ب م م السبيك

المعلم - القاهرة

٢٧٧٠٩١٤٠١٠٤٦٨١

KM6838788






جمهورية مصر العربية
بطاقة تحصيل النسخة

سماح
عبدالوازي محمود صالح
ب ٦ م ٢ اسبيكو
المعظم - القاهرة

KM6838788

٢٧٧.٩١٤.٠١.٤٦٨١



٢٠٢٤/٠٤ ٢٧٧.٩١٤.٠١.٤٦٨١

بدون عمل



أنثى مسلمة متزوجة
عبدالرحمن كمال عبدالرحمن محمد ابراهيم
البطاقة سارية حتى ٢٠٣١/٠٤/١٤





Presidential Initiative for Women's Health - Pathology Lab
مركز الرعاية الصحية للمرأة - مختبر الأمراض

Patient Name : *الاسم / اسم المريض*
Patient No. : 27709140104681
Path No. : 3728/23
Sex : Female
Age : 46 Y
Referring Doc :

Request No. : 13008911
Request Date : 29/10/2023
Verification Date : 06/11/2023
Financial Category : *نوع الخدمة*
Department : Out Patient
Registered by : marwa mohamed youssef

Pathology Report

Clinical data:
Left breast mass.

Nature of specimen:
Tru cut biopsy, collected at 29/10/2023 , 12:30 pm.

Gross examination
Multiple greyish white tissue cores up to 1.5 cm , totally submitted.

Microscopic examination:
* Histological type: Invasive duct carcinoma
* Grade (differentiation + nuclear pleomorphism + mitosis) . (3+2+1) Grade II .
* DCIS: Focal.
* Angioinvasion: Could not be detected within the limits of biopsy received.

Diagnosis:
Left breast mass, tru cut biopsy:
Invasive duct carcinoma, Grade II .
For further IHC (ER,PR,HER2,Ki67).

Prof. Gamal Abdelatty
Dr. Menan Hossam

Verified by :

Printing Date: 31/01/2024 11:44:20

Page 1 of 1

Printing By : Hiba Mayada Hegazy



Room No. (4) - First Floor Building No. 583, Nile Corniche - Miss H.Q. El-Dokki - Cairo, Egypt
مركز الرعاية الصحية للمرأة - مختبر الأمراض - 1987 - القاهرة - مصر - طابق الأرض - مبنى رقم 583 - شارع النيل - القاهرة - مصر

Patient Name : Sarah Abd Al Razek Mahmoud Safah	Date : 13/11/2023
Exam : C.T CHEST WITH CONTRAST	Age : 48 Year(s)
Medical ID: 4272/2023	Pat. ID: 43412

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED:-

1. Chest
 - No pulmonary nodules, masses or cavitary lesions.
 - No pathological hilar or mediastinal lymph nodes.
 - No evidence of pleural effusion, pleural thickening or masses seen.
 - Average cardiac size. No pericardial effusion detected.
 - Normal CT appearance of the thoracic aorta and the great vessels.
2. Abdomen and Pelvis
 - The liver is average in size with homogenous CT texture. No definite focal lesions or contour irregularities.
 - Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
 - No intra-hepatic or extra-hepatic biliary radicles dilatation.
 - Calculus gall bladder is seen.
 - Normal size & parenchymal texture of the spleen with no focal lesions seen.
 - No pathological abdominal or pelvic lymph nodes.
 - Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
 - Normal appearance of the pancreas, aorta, IVC and both suprarenals.
 - Slight filling of the urinary bladder with no filling defects or diverticular outpouching.
 - Bulky uterus with IUCD.
 - Left adnexal cyst measures 2 x 2.6 cm likely functional.
 - Normal appearance of the other pelvic organs.
 - No abdominal or pelvic masses or collections.
3. The scanned skeleton
 - The scanned skeleton shows no definite metastatic lytic or sclerotic lesions.
 - No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

- No pulmonary, hepatic or osseous metastatic deposits.

Dr. Wafaa Ali, FRCR

Signature Prof. Mervat Elallah Hamdy
21/11/2023 02:49:1



Patient Name: Samah Abd Al Razek Mahmoud Saleh	Date : 28/01/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 47 Year(s)
Medical ID: 4272/2023	Pat. ID: 43412

HIGH RESOLUTION ULTRASOUND OF BOTH BREASTS

Indication:

Left breast conserving therapy

Report:

- Scattered fibro glandular density (ACR B)

Left breast:

- Operative bed parenchymal distortion and scarring with a stationary large septated collection showing turbid content measuring about 50x40x30mm=35ml. No sizable mass at the operative bed
- No suspicious mass lesions or micro calcifications
- A few scattered turbid small cysts
- Marked diffuse edema pattern in the form of increased trabeculations and skin thickness.
- No regional lymphadenopathy

Right Breast:

- No suspicious micro calcifications or architectural distortion detected.
- No spiculated mass
- Normal retro-areola ducts
- Normal skin thickness, and nipple-areola complex
- Nonspecific axillary L.N.

Opinion:

- Post left BCS status showing complicated operative bed collection with no evidence of recurrence (BIRADS 2)
- Normal study of the right breast (BIRADS 1)

Best regards,

Dr. Amira H Radwan MD,
Breast Imaging consultant,
NCI, Cairo University
28/01/2025 2:17 PM

Patient Name: Samah Abd Al Razek Mahmoud Saleh	Date : 23/07/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 48 Year(s)
Medical ID: 4272/2023	Pat ID: 43412
1720	
1000	

MULTISLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS WITH CONTRAST

TECHNIQUE: Multi slice axial CT images of the chest, abdomen and pelvis were obtained followed by multiplanar reconstruction of the source images. IV contrast administration.

FINDINGS:

!!Known case of left breast cancer with back pain;

!!Reviewing the attached CT report dated on 16-11-2023;

(CCT CHEST):

- Left breast shows thickened parenchyma with UIQ shows elliptical lesion with central fluid density measures about 3.2x 1.8 x 3.7 cm in AP, TS & CC dimensions respectively
- No cavity lesions or bronchiectatic changes
- No CT evidence of hilar or mediastinal lymph node enlargement
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.



(CCT Abdomen and Pelvis):

- The liver is average in size showing homogenous CT density with no enhancing focal lesions or biliary radicles dilatation.
- The gall bladder shows average wall thickness. Its cavity shows dense stone.
- Normal CT appearance of the spleen, pancreas and both kidneys
- Regular filling of the urinary bladder showing no stones, masses or diverticulae
- No abdominal or pelvic pathologically enlarged lymph nodes
- No free collection.
- IUCD is seen.

The scanned skeleton:

Patient Name: Samah Abd Al Razek Mahmoud Saleh	Date : 23/07/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 48 Year(s)
Medical ID: 4272/2023	Pat. ID: 43412
1720	
1000	

- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.
- No vertebral structural collapse or extra-osseous soft tissue component
- Opinion:

"Reviewing the attached CT report dated on 16-11-2023;

- A Known case of left breast cancer still running stationary course regarding no pulmonary, hepatic or osseous metastatic deposit;
- Calculi gall bladder.

Dr. Heba El Balshy, MD

Lecturer of Diagnostic and Interventional Radiology,

MU-Menoufia University

Not Approved

Patient Name: Samah Abd Al Razek Mahmoud Saleh	Date : 23/07/2025
Exam : CT CAPIC (CT Chest Abd. Pelvis)	Age : 48 Year(s)
Medical ID: 4272/2023	Pat. ID: 43412
1720	
1000	

- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.
- No vertebral structural collapse or extra-osseous soft tissue component.
- Opinion:

**Reviewing the attached CT report dated on 16-11-2023:

- *A Known case of left breast cancer still running stationary course regarding no pulmonary, hepatic or osseous metastatic deposits.*
- *Calcular gall bladder.*

Dr. Heba El Balsby MD

Lecturer of Diagnostic and Interventional Radiology,

MT-Menoufia University

Not Approved



MDT form

1- Patient ID:

2- Name: معراج محمد الزاوي محمود شحات

Sex:

3- Medical number:
Female

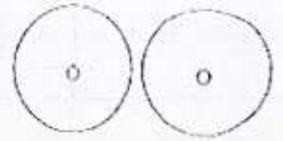
83172625

Old Med:

4272/2023

1-ID number: 27709140104661

Patient Age: 46 Y



Family history	Family history -VE	Phone	Phone 01281049500	BMI	BMI	REASON OF MDT	REASON OF MDT N/A
Marital status	Marital status MARRIED	NRS Score	NRS Score	GROUP	GROUP I	Date of MDT	Date of MDT 30/11/20
Co morbidity	Co morbidity HTN	Menopausal status	Menopausal status	Initial STAGE	Initial STAGE Early breast		

Clinical data					
Breast Imaging :			Mets Workup :		
T breast distortion 6 o'clock 12*13 mm BIRADS 4 no axillary LNS			NO pulmonary on hepatic deposits		
BX	ER	PR	Her 2 Neu	SISH	Ki67
RECOMBENT T breast mass TRUE CM	ER COMMENT +VE	PR COMMENT +VE	HER 2 Neu COMMENT -VE	SISH COMMENT	15% +VE
T		N		M	
		O			
Sonomammography					
NO MASS IN LEFT BREAST WITH NO LN					
PATIENT HISTORY					
MDT OUTCOME & PLAN					
FOR CEM --> IF UNIFOCAL FOR WLE& SLND (IN CLINIC)					

SIGNATURE			
MEDICAL ONCOLOGY	MEDICAL ONCOLOGY	RADIOLOGY	RADIOLOGY
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition

about black

Patient's Name:

Samah Abdelrazek Mahmoud

Date:

2026-05-04

Referred by:

Prof.Dr. CAPwithC

Hospital:

Goustave Roussey International Egypt

Findings:

- The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities.
- Gall bladder stone is seen measuring 2.5cm.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

Opinion:

- Gall bladder stone.
- No hepatic or bone metastasis.

Dr. Mai Shaaban MD

Patient's Name:

Samah Abdelrazek Mahmoud Saleh

Date:

2026-08-26

Referred by:

Prof. Dr. no doctor

Hospital:

Goustave Roussey International Egypt



Bilateral Digital soft tissue mammography with high resolution breast

sonography revealed:

Clinical summary: Post left CBS and ALAND followed by radiotherapy, routine follow up study.

Findings:

Right breast.

- Heterogeneously dense breast parenchyma that may obscure small lesions. (ACR c)
- No speculations, masses, asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Left breast.

- Heterogeneously dense breast parenchyma that may obscure small lesions. (ACR c)
- Thickened skin and coarsen trabeculae.
- Upper central focal asymmetry.
- LIQ architectural distortion with no speculations, masses, asymmetry, or suspicious microcalcifications.
- **Ultrasound** demonstrated LIQ area of parenchymal disruption underneath the operative scar with no solid , cystic lesions or fluid collections.
- Thickened skin reaching 5 mm and coarsen trabeculae.
- 12-1 o'clock, middle third, circumscribed oval parallel homogenous hypoechoic benign looking solid mass lesion measuring 9 x 5 mm with absent internal vascularity on color Doppler study.
- Normal nipple/areola complex.
- Clear left axilla. No pathological infra or supra-clavicular lymph nodes

Conclusion:-

Right breast (BIRADS 1): Normal study.

Left breast (BIRADS 3): Post-therapeutic sequelae & UOQ benign looking lesion

Recommendation:-

US- guided Tru-cut biopsy is advisable in light of clinical concern.

Best Regards,

DR. Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

Patient's Name:

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2026-05-04

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- No hepatic or bone metastasis.

Dr. Mai Shaaban MD

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Hospital:

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